

August 12, 2026

Absolute Total Care (Centene)  
Provider Appeals Department

**RE: Formal Appeal of Claim Denial — Eduardo Treviño (MRN MUSC4495205), Claim MUSC-B898-3**

|                          |                                                                                                                     |
|--------------------------|---------------------------------------------------------------------------------------------------------------------|
| <b>Patient</b>           | Eduardo Treviño (DOB 1971-02-01)                                                                                    |
| <b>MRN</b>               | MUSC4495205                                                                                                         |
| <b>Member ID / Group</b> | ABS836618976 / GRP-24383                                                                                            |
| <b>Claim number</b>      | MUSC-B898-3                                                                                                         |
| <b>Date of service</b>   | 2026-05-10                                                                                                          |
| <b>Procedure billed</b>  | CPT 99214 — Office/outpatient visit, established patient, moderate complexity                                       |
| <b>Primary diagnosis</b> | J06.9 — Acute upper respiratory infection, unspecified                                                              |
| <b>Denied amount</b>     | \$609.78                                                                                                            |
| <b>Denial code</b>       | CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer. |
| <b>Appeal deadline</b>   | 2026-09-07                                                                                                          |

Dear Appeals Review Committee:

**SUMMARY OF APPEAL**

MUSC Health is submitting this first-level appeal on behalf of established patient Eduardo Treviño (MRN: MUSC9177165, Member ID: ABS836618976) regarding the denial of Claim MUSC-B898-3 for services rendered on May 10, 2026, at MUSC Health University Medical Center. The claim was denied on June 9, 2026, under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying payer remark stating that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests full reversal and payment of the billed amount of \$609.78.

**CLINICAL BACKGROUND**

Mr. Treviño is a 55-year-old male with an active diagnosis of hyperlipidemia (E78.5, onset 2012), for which he has been maintained on simvastatin 10 mg with ongoing laboratory monitoring over multiple years. He presented on May 10, 2026, to an on-campus outpatient hospital setting (Place of Service 22) under the care of Robert K. Lindqvist, MD (NPI 1314059687) for evaluation and management of an acute upper respiratory infection (J06.9). The visit was coded as CPT 99214, reflecting an office or outpatient visit for an established patient with moderate complexity medical decision-making. Mr. Treviño has a documented history of recurrent upper respiratory conditions, including prior episodes of acute viral pharyngitis, viral sinusitis, and acute bronchitis, all previously coded under J06.9, establishing a relevant clinical pattern for this presentation.

## **BASIS FOR APPEAL**

The denial on medical necessity grounds is not supported by the clinical facts of this encounter. CPT 99214 is an appropriate and well-recognized evaluation and management code for an established patient presenting with an acute symptomatic illness. Acute upper respiratory infection (J06.9) is a legitimate, commonly treated diagnosis that warrants in-person physician evaluation, particularly in a patient with an active chronic condition such as hyperlipidemia and a history of recurrent respiratory illness. The selection of a moderate-complexity level of service is consistent with the clinical context: the provider must assess the acute complaint, consider the patient's active medication regimen including simvastatin, and determine whether the acute illness interacts with or complicates ongoing chronic disease management.

The payer's reliance on RARC N115, citing a Local Coverage Determination, does not render this service non-covered. LCDs govern coverage for specific procedures and diagnostic services; they do not categorically exclude medically necessary evaluation and management visits for acute illness in established patients. The assertion that documentation does not establish medical necessity is a conclusory statement that does not account for the clinical appropriateness of a physician visit for an acutely symptomatic established patient. MUSC Health requests that Absolute Total Care conduct a clinical review of the complete medical record for this encounter, including the physician's documentation of the presenting complaint, clinical assessment, and management plan, before upholding this denial.

## **REQUESTED ACTION**

MUSC Health respectfully requests that Absolute Total Care (Centene) overturn the denial of Claim MUSC-B898-3 and issue payment at the allowed amount of \$378.21 in accordance with the contracted rate for CPT 99214 rendered on May 10, 2026. Should the plan require additional clinical documentation from the rendering provider, Dr. Robert K. Lindqvist, MUSC Health will facilitate prompt submission of the complete encounter record. This appeal is submitted within the 90-day dispute window, ahead of the September 7, 2026 deadline, through the secure provider portal as directed on the Explanation of Payment.

Respectfully submitted,

**Angela R. Whitfield, RN, BSN, CPC**

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## **Enclosures**

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record